

UUID:D586806E-08E2-4C46-89B3-121BF0FF693
TCGA-SY-A9G0-01A-PR

Redacted



ICD O-3
Carcinoma, urothelial NOS
8120/3
Site ^{EGLE} Bladder, posterior wall
C67.4
^{with} Bladder, lateral wall
C67.2
JW 2/24/14

Surgical Pathology Report

Final

Surgical Pathology Report

DIAGNOSIS:

A-E. Bladder, prostate, and pelvic lymph nodes, excision:

Procedure: Radical cystoprostatectomy.

Tumor site: Right posterior lateral wall.

Tumor size: 2.5 x 2.0 x 1.2 cm.

Histologic type: Urothelial (transitional cell carcinoma), with nested pattern.

Associated epithelial lesions: None identified.

Histologic grade: High-grade, grade 3 (of 3).

Tumor configuration: Solid/nodule with ulceration.

Microscopic tumor extension: Perivesical fat.

Margins: Negative, right and left distal ureter (specimens C and D), and urethra.

Lymph-vascular invasion: Absent.

Perinodal invasion: Present.

Prostate: Extensive invasion by urothelial carcinoma.

Immunoperoxidase studies were performed on paraffin-embedded tissue.

These show that the neoplastic cells are reactive with antibodies to p63, but not cytokeratin 5/6, prostatic acid phosphatase or prostate-specific antigen. These findings support the diagnosis of extensive involvement by urothelial carcinoma rather than prostatic adenocarcinoma, as diagnosed in the review of previous outside material

Seminal vesicles: Negative for malignancy.

Lymph nodes (Specimens A and B): Metastatic carcinoma consistent with bladder primary, 1/3 left pelvic. Sections of the right pelvic lymph nodes show no evidence of metastatic carcinoma, four nodes.

Research tissue (block E22): Tumor (urothelial carcinoma) and normal prostatic tissue identified.

Pathologic staging: pT4, pN1.

Total number examined: Seven.

Number involved: One.

F. Kidney, right, radical nephrectomy: Marked chronic nephritis with hydronephrosis.

Permanent sections confirm the frozen section diagnoses.

CLINICAL INFORMATION:

Invasive bladder cancer.

SPECIMEN(S):

A:Right pelvic lymph nodes
B:Left pelvic lymph nodes
C:Left distal ureter margin
D:Right distal ureter
E:Bladder & prostate
F:Right kidney

PRELIMINARY INTRAOPERATIVE DIAGNOSIS:

Intraoperative Pathologist(s):

C. Left distal ureteral margin, frozen section diagnosis:
Negative for malignancy.

D. Distal right ureter, frozen section diagnosis: Negative for carcinoma.

E. Bladder and prostate, frozen section of distal urethral margin, frozen section diagnosis: Negative for carcinoma.

GROSS DESCRIPTION:

Performed by

A. Received fresh labeled and "right pelvic lymph nodes" are multiple pieces of adipose tissue, aggregating to 7.4 x 6.5 x 2.2 cm. Within the adipose tissue are several lymph nodes. All of the lymph nodes are submitted in cassettes labeled as follows: A1) One lymph node; A2) one lymph node bisected; A3) two lymph nodes, larger bisected.

B. Received fresh labeled and "left pelvic lymph nodes" are several pieces of adipose tissue, aggregating to 8.5 x 4.5 x 2.5 cm. Within the adipose tissue are several fatty lymph nodes. All of the lymph nodes are submitted in cassettes labeled as follows: B1) One lymph node bisected; B2) one lymph node bisected; B3) one lymph node bisected.

C. Received fresh labeled and "left distal ureteral margin" is a tubular segment of tan tissue consistent with ureter, 0.6 cm in maximum dimension. Entirely submitted for frozen section in one cassette labeled

D. Received fresh labeled and "right distal ureter" is a tubular segment of pink-tan tissue with attached fat, 1.2 cm in maximum dimension. Entirely submitted for frozen section in one cassette labeled

E. Received fresh labeled and "bladder and prostate" is a radical cystoprostatectomy specimen, 195 g and 10.5 x 6.2 x 4.2 cm. The prostate is separately 3.5 x 3.5 x 2.7 cm. The specimen is opened through the prostatic urethra and reveals a mucosal ulcerating mass in the right posterolateral wall, 2.5 x 2 x 1.2 cm. The mass extends into the bladder wall and possibly

extending up to and into the right portion of the prostate. The remaining bladder wall is edematous. The right and left ureteral orifices appear obstructed by the mass. Fresh tissue of the tumor, normal bladder, and prostate are submitted for

Multiple representative sections are submitted in cassettes labeled as follows: E1) Frozen section distal urethral margin; E2) anterior bladder wall; E3) bladder dome; E4) right lateral wall; E5) right lateral wall; E6) posterior wall; E7) posterior wall; E8) right posterolateral wall; E9) right ureteral bladder orifice; E10) left ureteral bladder orifice; E11) right and left seminal vesicles; E12) right prostate apex; E13) left prostate apex; E14) right prostate mid; E15) left prostate mid; E16) right prostate base; E17) left prostate base; E18-E20) junction of right prostate and right bladder wall; E21) possible parabladder lymph nodes; E22) mirror image of tissue corresponding to research IRB

F. Received fresh labeled and "right kidney" is a total nephrectomy specimen, 12 x 7.5 x 4.5 cm, 168 g. After the perirenal fat is removed, the kidney is 80 g and 9.2 x 3.8 x 2.5 cm.

The attached ureter is 15 x 0.6 x 0.6 cm. The capsule strips easily. The cortices are yellow-brown and granular. The cut surface of the kidney shows a grossly dilated renal pelvis with blunting of the cortices, changes consistent with hydronephrosis. The corticomedullary junction is distinct. There is no evidence of masses or cysts. The ureter is dilated which shows no evidence of tumor. Representative sections are submitted in cassettes labeled as follows: F1) Vascular and ureteral margins; F2-F3) sections of kidney; F4) additional sections of dilated ureter.

Support for the diagnosis in this case may have included the use of immunohistochemistry tests that were developed and whose performance characteristics were determined by

They have not been cleared or approved by the U.S. Food and Drug Administration (FDA). The FDA has determined that such clearance or approval is not necessary. These tests should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 ('CLIA') as qualified to perform high complexity clinical laboratory testing.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Notes		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	BDH	Date Reviewed: 12/11/2012